Warnings and Precautions (5.6) and Adverse Reactions (6.1)]. The 2.5 mg dosage is for treatment initiation and is not intended for glycemic control.

- After 4 weeks, increase the dosage to 5 mg injected subcutaneously once weekly.
- If additional glycemic control is needed, increase the dosage in 2.5 mg increments after at least 4 weeks on the current dose. The maximum dosage of MOUNJARO is:
  - 15 mg injected subcutaneously once weekly in adults.
  - 10 mg injected subcutaneously once weekly in pediatric patients.
- If a dose is missed, instruct patients to administer MOUNJARO as soon as possible within 4 days (96 hours) after the missed dose. If more than 4 days have passed, skip the missed dose and administer the next dose on the regularly scheduled day. In each case, patients can then resume their regular once weekly dosing schedule.
- The day of weekly administration can be changed, if necessary, as long as the time between the two doses is at least 3 days (72 hours).

2.2 Important Administration Instructions

- Inform patients and their caregiver(s) which MOUNJARO presentation (e.g., vial, pre-filled single-dose pen, single-patient-use KwikPen) they will receive and ensure they receive training appropriate for that specific presentation. If the prescribed MOUNJARO presentation changes, ensure patients and caregivers receive appropriate training and instruct them to consult the Instructions for Use for the newly prescribed presentation.
- Prior to initiation, train patients and their caregiver(s) on proper injection technique for the prescribed MOUNJARO presentation *[see Instructions for Use]*. After training, a patient may self-inject MOUNJARO if the healthcare provider determines that it can be properly administered, except for the following:
  - MOUNJARO KwikPen is not recommended for self-administration by pediatric patients.
  - MOUNJARO KwikPen is not recommended for self-administration by those who are visually impaired.
- Instruct patients using MOUNJARO vials to use a syringe appropriate for dose administration (e.g., a 1 mL syringe capable of measuring a 0.5 mL or 0.6 mL dose) and always use a new syringe and needle for each injection.
- Administer MOUNJARO once weekly, any time of day, with or without meals.
- Inject MOUNJARO subcutaneously in the abdomen, thigh, or another person should inject in the back of the upper arm.
- Rotate injection sites with each dose.
- Inspect MOUNJARO visually before use. It should appear clear and colorless to slightly yellow. Do not use MOUNJARO if particulate matter or discoloration is seen.
- When using MOUNJARO with insulin, administer as separate injections and never mix. It is acceptable to inject MOUNJARO and insulin in the same body region, but the injections should not be adjacent to each other.

3 DOSAGE FORMS AND STRENGTHS

Injection: Clear, colorless to slightly yellow solution in pre-filled single-dose pens, single-dose vials, multi-dose vials, or single-patient-use KwikPens, each available in the following strengths. The multi-dose vials and single-patient-use KwikPen each contain 4 doses:

Single-dose Pen or Vial
2.5 mg/0.5 mL
5 mg/0.5 mL
7.5 mg/0.5 mL
10 mg/0.5 mL
12.5 mg/0.5 mL
15 mg/0.5 mL

Multi-dose Vial (4 doses per vial)		
Dose per Injection	**Total Strength per Total Volume**	**Strength per mL**
2.5 mg/0.6 mL	10 mg/2.4 mL	4.17 mg/mL

1 EXECUTIVE SUMMARY

1.1 Recommendation on Regulatory Action

A) This Reviewer recommends the approval of exenatide as an adjunctive therapy to metformin, a sulfonylurea, or a combination of metformin and a sulfonylurea when they fail to achieve adequate glycemic control.

B) The one short-term controlled trial #2993-120 to support the use of exenatide as a monotherapy does not suffice as a proof of efficacy. This Reviewer recommends an action of "approvable" of exenatide as a monotherapy for the treatment of type 2 diabetes, pending evidence from at least one adequately controlled Phase III trial to support its use as a monotherapy in type 2 diabetes.

1.2 Recommendation on Postmarketing Actions

1.2.1 Risk Management Activity

The Risk Management Plan proposed by the Sponsor is acceptable to this reviewer.

Identified Risks
Exenatide treatment is associated with gastrointestinal side effects (nausea in 44%, vomiting in 13%, Diarrhea in 13% and dyspepsia in 6% of the treated subjects). Exenatide delays gastric emptying and may delay or decrease the absorption of concomitant drugs administered orally. When exenatide is used together with a sulfonylurea (SFU), it may increase the risk of SFU-induced hypoglycemia. Exenatide therapy is not associated with increased risk of hypoglycemia when used alone or in combination with only metformin. Forty-four percent of patients treated with exenatide develop anti-exenatide antibodies, but both the number of antibody-positive subjects and the titers of the antibodies in individual patients generally decrease as the length of exposure increases. Patients who developed anti-exenatide antibodies had similar rates and types of adverse events as those with no anti-exenatide antibodies. However, for the 14% of anti-exenatide-positive subjects (6% of the total exenatide-treated subjects) with higher antibody titers (1/625 to 1/15,625) still present at Week 30, glycemic responses were diminished for about half of the subjects while the rest had no apparent decrease in efficacy. Patients with diminished glycemic response to exenatide may therefore be patients with high anti-exenatide antibody titers.

Proposed Risk Management Activity
In addition to dealing with potential risks by using the appropriate language in the US Package Insert (USPI) and in the content of medical education instructional programs, a three-component risk management program will be established with: